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Name: أسماء محمد حسن عبدالله National ID: 28008091801741	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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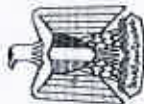
المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي CT scan of the chest.
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٠٢١/١٢ ٢٨٠٠٨٠٩١٨٠١٧٤١

حاصلة على بكتوريوس تجارة



مستلمة

أنثى

البطاقة سارية حتى ٢٠٢٨/١٢/١١



الجمهورية العربية السعودية
وزارة التعليم
بطاقة تحصيل الشخصية

الاسم
محمد حسن عبدالله أبو السعود
هـ ح الشكور حسن كامل
الساحل - القاهرة

٢٨٠٠٨٠٩١٨٠١٧٤١

JE1253231

DMS

٢٨٠٨٠٩١٨٠١٧٤١

Patient Name	: الأبيات / أسماء بنت محمد	Request No.	: 13009216
Patient No.	: (28008091001741	Request Date	: 29/08/2024
Path No.	: SR-852/24	Verification Date	: 08/07/2024
Sex	: Female	Financial Category	: فئة المرضى - Out Patient
Age	: 43 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youssef

Pathology Report

Clinical data
Left breast mass

Nature of specimen
Slide and block revision

Gross examination
One block labelled by (3364/24) and six slides were received.

Microscopic examination:

- Histological type: Invasive duct carcinoma.
- Grade (differentiation + nuclear pleomorphism + mitosis): (3+2+1) Grade II.
- ER: Positive
- PR: Positive
- Her2: Positive
- Ki67: Positive

- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8)
- PR: Moderate nuclear reaction in 80% of tumor cells (score 7/8)
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+)
- Cyt: Block Negative for metastatic deposits.

Diagnosis:

Left breast mass, slide and block revision
Invasive duct carcinoma, Grade II

- ER: Positive (score 8/8)
- PR: Positive (score 7/8)
- Her2: Positive (score 3+)

Verified by
Dr. Esraa Haggdy

Printing Date: 10/07/2024 08:55:55

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Printing By: Mrs. Mayada Haggdy

مركز الطبابة المرضية - مبنى مستشفى الملك عبد الله الثاني - شارع الملك عبد الله الثاني - عمان - الأردن
Phone: 011 555 5555 - Fax: 011 555 5555 - Email: info@moj.gov.jo



Presidential Initiative for Women's Health - Pathology Lab
 مركز المرأة لدراسة الأمراض - علم الأمراض



Patient Name : عبد الله بن محمد
Patient No : 28008001801741
Path No. : SR-852/24
Sex : Female
Age : 43 Y
Referring Doc :

Request No. : 13009216
Request Date : 29/06/2024
Verification Date : 08/07/2024
Financial Category : **أورام غير الليمفم - نكثة الورك**
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Cell block. Negative for metastatic deposits.

Prof. Lubna Omar
Dr. Hiba Hassan

Verified by:
dr. eszra magdy

Printing Date : 10-07-2024 08:56:52

Page 2 of 2

Printing By: Mrs. Mayada Hegazy

[illegible]

PATIENT NAME: ASMAA MOHAMMED Hassan ABDALLA
GENDER: Female
DATE : 18-Jul-2024 11:41 AM

PATIENT ID: 10124243357
AGE: 43 y
ORDER: 2944117

99MTC-MDP BONE SCAN

Desired Information: •
skeletal survey.

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

No hot spot that account for osseous deposits could be detected all over the surveyed skeleton.
Degenerative changes are seen over dorso-lumbar vertebrae.
Arthritic changes are seen over both knees.
Both kidneys are normally visualized.

Conclusion:

No definite evidence of metastatic osseous deposits.

Reported By,

Prof. Huda Fathy, Nuclear Medicine Consultant

18-Jul-2024 11:05 AM

PATIENT: ASMAA MOHAMMED Hassan ABDALLA, 10124243357, Female, 43 y
Report version 1, 18-Jul-2024 11:05 AM

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مركز
التصوير
الاشعاعي
بمبنى
الطوارئ
بمستشفى
الشيخ زايد



مركز
التصوير
الاشعاعي
بمبنى
الطوارئ
بمستشفى
الشيخ زايد

Name: Mrs. Asmaa Mohamed Hassan Abdullah

Date: 12/12/2024

ID:3316012125

Acc.:240207074

M-Mode Measurements:

LVEDD	49 mm	(37-56)	LVESD	31 mm	
SWT	8 mm	(7-11)	PWT	7 mm	(7-11)
LA	41 mm	(< 40)	AOR	34 mm	(24-37)
EF	66%	(52-72 %)	FS	36%	(23-43 %)

Comments:

- Sinus rhythm.
- The left ventricle is grossly normal size, there is normal left ventricular wall thickness, left ventricular systolic function is normal. Ejection fraction by M-Mode = 66%, the same by 2D eyeballing. The trans mitral spectral Doppler flow pattern is suggestive of normal LV diastolic function. No SWMA could be detected at rest.
- The right ventricle is normal in size and function.
- The left atrial size is mild dilated, right atrial size is normal, normal inferior vena cava size and collapsibility suggests normal right atrial pressure. The inter-atrial septum is intact with no evidence of atrial septal defect.
- The mitral valve is normal, there is trace to mild mitral regurgitation.
- The tricuspid valve is normal there is trace tricuspid regurgitation, with estimated RVSP = 21 mmHg.
- Aortic valve is tri-leaflet, normal in structure and function.
- The pulmonic valve is normal in structure and function. There is trace pulmonic valvular regurgitation.
- The aortic root is normal in size.
- There is no pericardial effusion.

Conclusion:

The left ventricular systolic and diastolic functions are normal.

Signature;

Dr. Moustafa Fathy Mohamed
Cardiology, MD

Scanned by
Tahra Radiology Center

TAHRA RADIOLOGY CENTER
1000 EL DOKKI, CAIRO, EGYPT



TAHRA

RADIOLOGY

CENTER
1000 EL DOKKI, CAIRO, EGYPT

Name: Mrs. Asmaa Mohamed Hassan Abdullah

Date: 14-12-2024

ID:3316012125

Acc.:5060497

Digital 3D soft tissue mammography & tomosynthesis with ultrasound examination of both breasts, revealed:

Indication: known case of pathologically proved left breast invasive ductal carcinoma grade III (ER positive, PR positive & Her 2 positive) received radiotherapy & chemotherapy for follow up.

Q

Mammography & Tomosynthesis:

- Heterogeneous dense breast parenchyma (ACR-c)
- **Left breast Compared to pre-therapeutic images:**
 - No gross evidence of dense spiculated lesion or parenchymal distortion
 - Clip is noted at the inner central middle third 3.3 cm distance from the nipple
 - Retro-areolar tubular branching isodense lesion
- **Right breast:** No gross evidences of dense spiculated masses or architecture distortion seen
- The tomosynthesis slices showed no abnormality
- No suspicious micro-calcific clusters noted
- Normal skin thickness and contour of both sides
- Normal areola / nipple complexes on both sides

Ultrasonographic examination showed:

- Heterogeneous fibroglandular (breast) parenchyma with predominant glandular tissue associated with few scattered tiny cystic lesions suggestive of fibrocystic changes
- **Left breast:**
 - No gross evidence of suspicious lesions or residues
 - Single level axillary lymph node with focal thickness 0.38 cm (indeterminate)
 - Retro-areolar dilated ducts with clear anechoic content corresponding to the retro-areolar isodense lesion in mammography
 - Ultrasound couldn't detect clip definitely, for stereotactic wire localization.

Name:

Mrs. Asmaa Mohamed Hassan Abdullah

Right breast:

- Retroareolar well-defined oval shaped lesion measuring 5 x 3 mm in average diameter, parallel orientation, no posterior features, no internal vascularity
- No gross evidence of suspicious solid masses
- No retro-areolar dilated ducts noted
- Normal skin thickness & contour. Normal nipple areola complex.
- Bilateral non-specific axillary lymph nodes with preserved oval shaped and central echogenic hilum

Opinion:

- **Left breast:** known case of left breast pathologically proved invasive duct carcinoma grade II showing features suggestive of complete radiological response as described BIRADS 6
- **Right breast:** mild fibrocystic changes with retro-areolar probably benign lesion as described (BIRADS 2)

Recommendation:

- Stereotactic Wire localization is recommended

Prof. Dr. Corina Adel, MD

Female / Breast Imaging & Intervention Consultant

Cairo University

BIRADS scoring is the breast imaging and reporting data system for classifying the radiological diagnosis of breast lesions:

- **BIRADS 0:** Incomplete - need additional imaging, evaluation or comparison with previous films
 - **BIRADS 1:** Normal examination - nothing to concern on - Routine mammography screening is recommended
 - **BIRADS 2:** Benign findings - routine mammography screening is recommended
 - **BIRADS 3:** Probably benign (<2% malignancy) - short interval follow up is suggested
 - **BIRADS 4:** Suspicious (2-20% malignancy) - Biopsy should be considered
 - 4A Low suspicion of malignancy - 2-20%
 - 4B Mod suspicion of malignancy - 2-20%
 - 4C High suspicion of malignancy - 8-20%
 - **BIRADS 5:** Highly suspicious of malignancy (50%) - appropriate action should be taken
 - **BIRADS 6:** Known - biopsy proven malignancy - Surgical excision when clinically appropriate
- American college of radiology (ACR) scoring of breast density:**
- **ACR a:** almost entirely fatty
 - **ACR b:** scattered areas of fibro-glandular density
 - **ACR c:** heterogeneously dense
 - **ACR d:** extremely dense

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مركز التصوير الطبي
مركز التصوير الطبي
مركز التصوير الطبي

Name: Mrs. Asmaa Mohammed Hassan Abd-Allah

Date: 1/5/2025

ID:3316012125

Acc.:240232065

M-Mode Measurements:

LVEDD	46 mm	(37-56)	LVESD	27 mm	
SWT	7 mm	(7-11)	PWT	8 mm	(7-11)
LA	41 mm	(< 40)	AOR	28 mm	(24-37)
EF	72%	(52-72 %)	FS	41%	(23-43 %)

Comments:

- Sinus rhythm
- The left ventricle is grossly normal size, there is normal left ventricular wall thickness, left ventricular systolic function is normal. Ejection fraction by M-Mode = 72%, the same by 2D eyeballing. The trans mitral spectral Doppler flow pattern is suggestive of normal LV diastolic function. No SWMA could be detected at rest.
- The right ventricle is normal in size and function.
- The left atrial size is mild dilated, right atrial size is normal.
- The mitral valve is normal, there is mild mitral regurgitation.
- The tricuspid valve is normal there is mild tricuspid regurgitation, with normal estimated RVSP.
- Aortic valve is tri-leaflet, normal in structure and function.
- The pulmonic valve is normal in structure and function. There is trace pulmonic valvular regurgitation.
- The aortic root is normal in size.
- There is no pericardial effusion.

Conclusion:

The left ventricular systolic and diastolic functions are normal.

Signature:

Dr. Moustafa Fathy Mohamed
Cardiology, MD

Dr. Moustafa Fathy Mohamed
Cardiology, MD

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Code F100-02-2021

Dr. Moustafa Fathy Mohamed
Cardiology, MD



Dr. Moustafa Fathy Mohamed

Cardiology, MD

Dr. Moustafa Fathy Mohamed
Cardiology, MD

Patient Name : Dr. Heba Hassan	Request No. : 13007637
Patient No. : 28008091801741	Request Date : 22/12/2024
Path No. : 5131/24	Verification Date : 30/12/2024
Sex : Female	Financial Category : Upt 00 - Out Patient
Age : 44 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs. Mayada Hagazy

Pathology Report

Clinical Data : Right breast cancer - post NACT

Nature of the specimen : Right WLE & Right SLN biopsy

Gross Examination

- Rt WLE specimen measured 7 x 6 x 3 cm - covering skin ellipse measured 5 x 1.5 cm
- Cut section revealed - well defined greyish white firm tumour bed site measured 5 x 2.5 x 2.5 cm away from (1, 0.4, 1.0, 2.0, 4, 3) cm from (superior, inferior, lateral, medial, deep & skin) margin.
- SLN biopsy : A fibrofatty tissue measured 3x3 cm enclosing 4 SLNs
- Frozen section: Free margins & free SLNs (0/4)

Microscopic examination

- Histological type: No residual tumor
- DCIS: No
- Angiolymphatic: No
- Lymph node involvement: Free SLNs (0/4)
- Resection Margins: Free margins
- Histologic response to therapy: Pathologic complete response

Diagnosis:

- Right WLE & Right SLN biopsy
- No viable tumor in a previously diagnosed invasive duct carcinoma
- Free margins
- Free SLNs (0/4)
- Pathologic complete response
- Stage : ypT0 , ypN0

Verified by
Dr Heba Hassan

Printing Date : 11/01/2025 09:57:26

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Printing By : maswa.mohamed.youssef



المرجع البيولوجي للإمرأة والصحة - هليوبليس - طابق الأرض - مبنى 78 - ساحة كورنيش - مصر 11431 هليوبليس - القاهرة، مصر
Room No. 111 - First Floor Building No. 78 - Side Corniche / Most El Qasr El Heliopolis - Cairo, Egypt

Acc: 7401,0395

HITLER'S CHIEFS

- Right pleural effusion posteriorly superiorly
- Mild rightward shift of the trachea
- Otherwise clear both lung fields and no other abnormalities detected. No pneumonia, consolidation, cavitation or bronchovascular changes seen
- No evidence of pleural space collection
- No evidence of enlarged hilar or mediastinal lymph nodes
- No evidence of enlargement of the heart or pulmonary vessels

Right upper lobe posterior segment angle peripheral fibrotic band with thin right pleural thickening, likely inflammatory in nature, for close follow up. Otherwise normal CT chest.

- External morphology by extending surface via a 2° cleft in pelvis
- Normal CT appearance of the spleen (pancreas): Splanchnic ganglia, not visible in CT, V3 and some organs
- Anterior wall is even
- No CT evidence of enlarged lymphatic nodes by splenectomy
- No calcifications
- No evidence of peritoneal effusions
- No calcifications

- Anti-hepatocarcinogenic

- c. Oblique. Normal CT examination of the abdomen and pelvis.

Trudy Jones.

Dr. Mohamed-Elah El-Sayed, *et al.*

PATIENT NAME: ASMAA MOHAMMED Hassan ABOALLA

GENDER: Female

DATE: 17-Jul-2024 01:09 PM

PATIENT ID: 1012424330

AGE: 43 y

ORDER: 2044110

CONTRAST ENHANCED SPECTRAL MAMMOGRAPHY**INDICATION:**

• left breast mass

TECHNIQUE:

- Contrast Enhanced Spectral Mammography CC and MLO views
- IV non-ionic contrast medium infusion 100 ml of omnipaque

REPORT:

- Contrast Enhanced Spectral Mammography showed mild symmetrical back ground parenchymal enhancement.
- Left breast retroareolar non mass enhancement showing moderate heterogeneous enhancement measured about 36 x 28mm with posterior linear enhancement measuring collectively with the mass about 44 x 37mm(measured in MLO view).
- Few anterior faintly enhancing nodule extending to the NAC by US correlation confirmed to be prominent ducts with echogenic contents extending from the mass to NAC likely ductal extension.
- Right breast retroareolar small mass enhancement confirmed by US as 12.00 supra-areolar small hypoechoic lesion with fat central contents likely L.N. it measures about
- Normal skin thickness and contour.
- Normal nipple areola complexes.

OPINION:

• Left breast pathological mass with suspicious enhancement with posterior and anterior probably ductal extensions(BIRADS 5).
• Right breast 12.00 small partially enhancing mass confirmed by US to be likely nodal for follow up (BIRADS 3)

Clinic

Reported By,

Dr. Sara Sakr, MSc

17-Jul-2024 01:00 PM

PATIENT INFORMATION: ASMAA MOHAMMED Hassan ABOALLA, 43 y
FEMALE, 17-Jul-2024 01:09 PM, 1012424330



PATIENT NAME: ASMAA MOHAMMED Hassan ABDALLA

GENDER: Female

DATE: 09-Jun-2024 03:31 AM

PATIENT ID: 10124243357

AGE: 43 y

ORDER:

2879113,2879114,2880802

**FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY
ULTRASOUND****INDICATION:**

- Left breast palpable lesion.

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

- Scattered fibroglandular densities (ACR B).
 - Left UOQ para areolar area of architectural distortion with overlying few calcific foci.
 - Left UOQ area of focal asymmetry.
 - Right para areolar small circumscribed iso dense nodule.
- Complementary Ultrasound showed:
- Left breast para areolar area of parenchymal distortion mounting to mass formation with multiple cystic changes / duct dilatation as well as dilated ducts, and internal vascularity, such lesion collectively about 42x15mm. Associated anterior and posterior duct dilatation ?? ductal extension with retroareolar duct dilatation, such lesion extends between 10 and 12 O'clock. small hypoechoic nodule about 3 mm at 12 O'clock about 50 from nipple.
 - Left UOQ 1 O'clock focal adenosis with internal cystic changes (corresponding to the forementioned area of asymmetry about 50x30 mm).
 - Right retroareolar small lobulated hypoechoic nodule with mild internal vascularity about 7x6 mm at 9 O'clock (?? small fibroadenoma VS LN).
 - Normal skin thickness.
 - Bilateral prominent fibroglandular tissue with tiny nodularity in keeping with adenosis.
 - Normal nipple areola complex.
 - Right nonspecific axillary lymph nodes.
 - Left axillary multiple enlarged LN with preserved hilum likely non

PATIENT: ASMAA MOHAMMED Hassan ABDALLA, 10124243357, Female, 43 y
Report Version 3, 09-Jun-2024 10:51 AM

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